



Behavioral

Assessment

Pediatric Pearls:

- Use pediatric dosing of medications or electrical therapy for a pediatric patient < 37 kg and as defined by HandTevy

Signs & Symptoms:

- Anxiety, agitation, confusion
- Affect change, hallucinations
- Delusional thoughts, bizarre behavior
- Expression of suicidal homicidal thoughts
- Tachycardia, diaphoresis, tachypnea
- Combative / violent

Differential:

- Refer to Altered Mental Status
- Alcohol intoxication
- Bipolar (manic-depressive)
- Schizophrenia, anxiety disorders, etc.
- Hypertensive emergency
- Seizure / Postictal
- Domestic Violence or Abuse

Clinical Management Options

P	P	P	P	P	P		• Oxygen, target SpO₂ 92 – 96% • BGL Assessment • Temperature assessment • Verbal de-escalation • Basic Airway Management as needed • Physical restraint, if needed and use sedation checklist • Cooling measures, if needed • Patient seated or head elevated 30°
L	L	L	L	L	L		• If sedated, place continuous cardiac, ETCO₂, and SpO₂ monitoring
1	2	3	4	5	6		• Vascular access as appropriate for patient condition • Fluid therapy as needed with isotonic crystalloid
							• 12 ECG • RASS +4 Hyperactive Delirium with Severe Agitation COG • RASS +2 or +3 (aggressive behaviors requiring sedation) Midazolam or Droperidol • RASS +1 or +2 (uncontrolled anxiety) Midazolam • Advanced Airway Management as needed
INTEGRATED SERVICES							• RASS +1 or +2, uncontrolled anxiety, agitation, acute schizophrenia, or bipolar disorder crisis, Olanzapine and connect to mental health resources within 24 hours

Consult Online Medical Control as Needed

Modified Out-of-Hospital RASS

- +4 Violent/ Immediate danger & unable to be consistently restrained
- +3 Aggressive movement: Offensive behavior directed at people, most “pushing” type movement
- +2 Vigorous movement: Most “pulling” type movement / defensive behavior
- +1 Restless: Uncontrolled Anxiety, movements not aggressive or vigorous
- 0 Alert and Calm: Spontaneously pays attention to provider
- 1 Drowsy: Not fully alert but sustains more than 10 seconds wake, with eye opening in response to verbal command
- 2 Light sedation: Awakens briefly for less than 10 seconds with eye contact or verbal command
- 3 Moderate sedation: Any movement, except eye contact, in response to command
- 4 Deep sedation: No response to voice, but any movement to physical stimulation
- 5 Unarousable: No response to voice or physical stimulation



Behavioral

Pearls:

- Refer to drug formulary charts for all medication dosing for both adults and pediatric patients.
- Consider your safety first. Physical restraint should be performed/assisted by Law Enforcement when available.
- SAVE Mnemonic for De-Escalation:
 - Support - "Let's work together..."
 - Acknowledge - "I see this has been hard for you..."
 - Validate - "I would probably be reacting the same way if I was in your shoes..."
 - Emotion naming - "You seem upset..."
- All patients who receive either physical restraint or sedation must be continuously monitored by ALS personnel on scene or immediately upon their arrival. Monitoring must include:
 - Cardiac
 - Pulse oximetry
 - ETCO₂ monitoring

This does not apply if the patient is restrained for law enforcement purposes only and law enforcement is immediately available, e.g., the transport of a prisoner in law enforcement custody who is not a behavioral/HDSA patient.

- Any transported patient handcuffed or restrained by Law Enforcement should be accompanied by an officer
 - handcuffed patients must never be handcuffed with their arm(s) behind them.
 - Patients may never be handcuffed to the stretcher, other parts of the vehicle, or other pieces of equipment
- Be sure to consider all possible medical and trauma causes for behavior.
- Restrained patients must never be maintained or transported in a prone position.